Control Quest · Study notes

NCM 433 · Controlling · Based on 5. Controlling (1).pdf, pp. 1–3.
Source: Joel Boniao | BSN4-N4. Memory hooks and practice prompts created for this study guide.

01. The big picture

Controlling closes the gap between the plan and what actually happens. (Handout p. 1)

Controlling means assessing and regulating performance against adopted plans, issued instructions, and established principles. It tells managers how well people and processes are working so they can support better performance.

Same purpose, different timing

BEFORE

Feedforward

Monitor inputs and anticipate problems before they affect the system.

DURING

Concurrent

Monitor activities as they happen and check that they meet standards.

AFTER

Feedback

Evaluate completed activities and use the results to improve future work.

> **Memory hook · Before, during, after.**
> Feedforward looks ahead. Concurrent stays alongside. Feedback looks back.

Imagine one ward shift

- **Before:** check whether supplies are ready for the shift → feedforward.
- **During:** observe practice while care is happening → concurrent.
- **After:** review last month's completed reports to improve the next month → feedback.

These ward examples were created to help apply the handout's definitions.

02. Evaluate with purpose

A useful evaluation makes improvement specific, visible, and fair. (Handout p. 1)

Why conduct evaluation?

- Ensure quality nursing care and set sensible objectives that staff can meet.
- Provide standards for comparison and help employees monitor their own performance.
- Highlight quality problems and areas needing priority attention.
- Show the costs of poor quality and justify the use of resources.
- Provide feedback that supports improvement.

Principles to remember

- Base evaluation on **behavioral performance standards** and allow enough time to observe behavior.
- Give employees their job description, performance standards, and evaluation form.
- Include both **satisfactory and unsatisfactory** results.
- Prioritize areas needing improvement.
- Schedule the conference at a convenient time for both people.
- Structure the evaluation report and conference.

Four qualities of an evaluation tool

Quality	What the handout emphasizes	Recall cue
Objectivity	Free from bias.	Fair judgment.
Reliability	Accurate. Think dependable, consistent measurement.	Dependable result.
Validity	Relevant to what is being evaluated.	Measures the right thing.
Sensitivity	Detects fine differences among the criteria measured.	Notices small differences.

> **Memory hook · O–R–V–S**
> Objective, Reliable, Valid, Sensitive: fair, dependable, relevant, detailed. The recall cues expand the handout's short labels.

03. Close the control loop

Set the target, measure reality, compare, and act. (Handout p. 1)

1 **Establish standards, objectives, and measurement methods.**
Decide what good performance looks like and how to assess it. The handout lists task analysis and quality control as methods for measuring nursing care.

2 **Measure actual performance.**
Use an ongoing, repetitive process. Frequency depends on the activity. Measurement may be scheduled or occur at periodic, unannounced intervals.

3 **Compare results with standards and objectives.**
Identify whether actual performance matches expectations.

4 **Reinforce strengths and take corrective action.**
Keep effective performance going and address gaps to improve results.

What exactly are we measuring?

THE SETUP

Structure

The agency's structure or management system for delivering care: staffing, equipment, resources.

THE ACTIONS

Process

What the nurse does in relation to the nursing process: how care is performed.

THE RESULTS

Outcome

What results from the care provided: changes in patient outcomes.

Memory hook · Setup → actions → results.
Enough IV pumps? Structure. Was a procedure carried out as expected? Process. Did fall rates decrease? Outcome.

04. Appraisal that helps

Performance appraisal compares an employee's work with standards over a specified period. (Handout p. 1)

What is appraisal for?

It reflects how well nursing personnel have performed during a defined time. The handout lists these purposes:

- Determine salary standards; select qualified employees for promotion; identify unsatisfactory performance for possible demotion.
- Make an inventory of talent and determine training needs.
- Improve work-group performance and communication.
- Establish supervisory performance standards.
- Discover employees' aspirations, recognize accomplishments, and explain where they stand.

Make the standard visible

- Base appraisal on a standard.
- Let the employee contribute to developing the standard.
- Ensure the employee knows the standard **in advance** and knows the sources of appraisal data.
- Use an appraiser who has **observed the work** and whom the employee trusts and respects.

Informal vs. formal appraisal

Informal	Formal
Incidental observation during nursing care.	Regular, methodical collection of objective facts.
A manager notices that a nurse missed hand hygiene during care.	A scheduled chart audit checks documentation accuracy and completeness against expectations.

> **Ask: incidental observation or organized review?**
> Both can identify performance issues. The difference is how the appraisal is conducted.

05. Tools, without the bias

Recognize the tool being used—and the judgment trap that can distort it. (Handout p. 2)

Appraisal tool	How to recognize it
Essay	A free-form narrative of strengths and areas needing improvement.
Ranking	Orders employees relative to coworkers, with respect to qualifications.
Checklists	A compilation of expected nursing performances or behavior descriptions.
Rating scale	Rates tasks or desired behaviors, including their presence, absence, or extent.
Forced-choice comparison	Selects statements that best or least describe the nurse from groups of favorable and unfavorable statements.
Anecdotal recording	A descriptive account of the nurse's experience with a person or group; can validate technical skills and interpersonal relationships.
Peer review	Colleagues of the same profession, rank, and setting evaluate performance against accepted standards.

Three checklist forms

- **Weighted scale:** behavioral statements representing desirable job behaviors.
- **Forced checklist:** select a desirable and an undesirable behavior for each employee.
- **Simple checklist:** check words or phrases that describe the employee's traits or behaviors.

Spot the appraisal pitfalls

Halo effect

One or two positive behaviors influence the assessment of everything else.

Horns effect

Negative aspects overshadow other areas, making the overall assessment inaccurate.

Central tendency

The appraiser avoids a true assessment and rates everyone as average.

Memory hook · Halo glows, horns darken, center flattens.
A standout positive or negative should not replace assessment of each performance area. Everyone-average ratings hide real differences.

06. Quality, checked

Quality assurance evaluates care; improvement uses what we learn to make it better. (Handout p. 2–3)

Four ideas that fit together

- **Quality assurance:** evaluation applied to the healthcare system and the services delivered by health workers.
- **Quality improvement program:** continuous measurement and evaluation of structure, process, and outcome. Example: monitor hand hygiene and implement strategies to reduce infections.
- **Benchmarking:** compare products or services with the best-performing organizations. Example: compare patient satisfaction with top institutions.
- **Quality of care:** the degree to which services increase the likelihood of desired health outcomes for individuals and populations.

Nursing audit documents the quality of nursing care in relation to the nursing department's established standards.

Audits have two separate lenses

Lens	Type	Clue
When?	Concurrent patient care audit	Care or charts are reviewed while the patient is still admitted.
	Retrospective patient care audit	Review after discharge: charts, questionnaires, or interviews.
What?	Structure audit	The setting: finances, staffing structure, records systems, equipment, environment.
	Process audit	How care was carried out; deviations from accepted best-practice process standards.
	Outcome audit	Results of nursing interventions: fall rates, infection rates, recovery outcomes.

> **Don't mix up timing with focus.**
> A review may be *concurrent* because the patient is admitted and focus on *process* because it checks how care is delivered. Read what the question asks.

The handout describes outcome audits as the most valid indicators of quality care. Keep that phrasing associated with the handout.

People improve quality, too

- **Peer review:** collegial performance evaluation to promote practice excellence.
- **Quality circles:** workers doing similar work voluntarily meet regularly during normal working time under their supervisor's leadership.

07. Discipline with direction

Discipline is a constructive way to build responsibility for performance and behavior. (Handout p. 3)

What strengthens self-discipline?

- Commitment to the institution's vision, philosophy, goals, and objectives.
- Professional laws and codes of conduct.
- Understanding agency rules and regulations.
- An atmosphere of mutual trust and confidence.
- Peer and organizational pressure.

Build a sound disciplinary program

Tailor the program to the institution's objectives. Include **policies and procedures, uniform application of rules, a disciplinary committee, and orientation for new employees.**

Six principles of successful implementation

Principle	Meaning
Promptness	Address the violation promptly, rather than weeks later.
Fairness	Match the response to the severity of the offense.
Impartiality	Apply rules consistently without favoritism.
Non-punitiveness	Aim for correction and improvement rather than punishment.
Advance warning	Tell staff the rules and possible consequences beforehand.
Follow-through	Enforce the rules consistently; do not ignore recurring violations.

> **Memory hook · Now, fair, equal, improve, inform, follow.**
> Promptness, fairness, impartiality, non-punitiveness, advance warning, follow-through.

08. The discipline ladder

Learn the progressive sequence described in the handout, including what each step contains. (Handout p. 3)

1 **Counseling and oral warning**
Discuss expectations for improvement, ways to correct the problem, and the possibility of further action if it happens again.

2 **Written warning**
State the problem and violated rule, consequences of continued behavior, the employee's commitment to corrective action, and planned follow-up.

3 **Suspension**
For a minor violation, the handout places suspension after evidence of oral and written warnings. It describes suspension rather than dismissal when management believes the employee can still be rehabilitated.

4 **Dismissal**
The handout places this after other disciplinary efforts fail. The committee must ensure the cause conforms to major-violation criteria in the policy manual.

> **Memory hook · Talk → write → suspend → dismiss.**
> This is the sequence taught in your handout. The policy manual and the committee's criteria remain part of the handout's dismissal requirements.

Read the details in a case

- Advice, improvement expectations, and a warning about repetition → counseling/oral warning.
- Documented problem, violated rule, consequences, commitment, and follow-up → written warning.
- Earlier warnings documented and improvement still considered possible → suspension in the handout's progression.
- Other efforts failed and policy criteria confirmed → dismissal in the handout's progression.